

Manager guidance — impairment at work

Who this is for. Anyone at (organisation) who supervises other people. It supports sections 5, 9 and 10 of the drug and alcohol policy — if this guidance and the policy ever appear to differ, the policy applies.

1. Your role — and its limits

You are not expected to diagnose anything, prove anything or run an investigation. Your role has exactly three parts: notice, act on safety, and escalate. Everything else — testing, medical review, disciplinary decisions — belongs to other people under the policy.

2. Noticing possible impairment

Signs that may indicate impairment include:

- smell of alcohol; slurred or unusually slow speech
- unsteadiness, poor coordination, drowsiness
- unusually erratic, aggressive or withdrawn behaviour
- glazed or bloodshot eyes, unusually large or small pupils
- a marked change in work quality, timekeeping or judgement

None of these is proof. Every one of them has innocent explanations — medication, fatigue, illness, diabetes, stress, a neurological condition. That is exactly why the policy uses testing and medical review rather than a manager's opinion. Your observation starts the process; it never concludes it.

3. Act on safety first

If you reasonably believe someone may be unfit for duty:

1. Remove them from the work calmly and without public discussion — especially from safety-critical duties such as (safety-critical list), and from any driving or equipment operation.

"safety-critical duties such as (safety-critical list)" is shown only when safety-critical categories are selected; otherwise the step reads "especially from any driving or equipment operation".

2. Do not let them drive. If they are stood down, (organisation) helps arrange safe transport home — this is a duty of care, not a punishment, and the policy says so.
3. Treat the stand-down as precautionary and say so: "I need you to stop work while we sort this out" — not "you're in trouble".

4. Holding the conversation

Do it privately, with a second manager or HR present where possible. Keep it short and factual.

- **Describe, don't accuse.** "I've noticed X and Y this morning" — not "you've been drinking".

- **Ask an open question.** “Is there anything going on that I should know about — anything affecting you at work?”
- **Listen for the two answers that change everything:** a medication declaration (handled under section 7 of the policy — confidential, no condition disclosed) and a voluntary disclosure of a problem (which the policy treats as a health matter, met with support).

“(which the policy treats as a health matter, met with support)” is shown only when the policy stance is support first.

- **Do not promise outcomes.** Not “it’ll be fine”, not “this means dismissal”. The process decides outcomes, not you.
- **Write down what you observed** — factually, at the time, in your own words. Behaviour, times, words used. Your contemporaneous note is what protects both of you later.

Section 5 is shown only when the selected testing types include for cause:

5. Arranging a for-cause test

A for-cause test may be arranged where there is a reasonable belief, based on observed behaviour, appearance or other evidence, that someone may be under the influence.

1. **Record the grounds first.** Before any test is arranged, write down what was observed, by whom and when. A test arranged without recorded grounds is the single most common way workplace testing fails.
2. **Get a second opinion where practicable** — another manager or HR confirming the observation strengthens the grounds and removes any suggestion of one person’s grudge.
3. **Contact (provider)** on (provider contact number) to arrange collection. Tell the individual a test is being arranged and why, in the terms recorded.

The contact number is shown only when the provider is NIVHA Laboratory Services, and renders as 02890 737942.

4. **The individual will be told, before deciding, how a refusal is treated** — that is the collector’s script, but do not contradict it. Never suggest refusing “won’t matter”.
5. **Do not collect anything yourself.** No sniffing drinks bottles, no searching bags, no photographs, no improvised breathalysers. Collection is done under chain of custody by trained collectors, or the result is worthless.

While the result is pending, the person remains stood down on full pay. A screening result that is not negative (“non-negative”) is not a positive — it goes to the laboratory for confirmation and medical review. Do not treat it, or describe it, as a fail.

6. What never to do

- Never diagnose (“he’s clearly an alcoholic”) — in writing or otherwise.
- Never discuss one person’s situation with colleagues, or confirm a test happened.
- Never search a person, their bag or vehicle — possession concerns go to (policy owner).
- Never delay a safety decision to be kind. Removing someone from duty protects them too.

- Never handle a medication disclosure as gossip — it is special category data under section 12 of the policy.

7. Afterwards

Whatever the outcome — negative result, confirmed violation, or support plan — the person returns to a normal working relationship. Where a return-to-work plan includes monitoring testing, that plan is agreed and confidential; your role is only to support the agreed adjustments. If you are unsure about anything under this guidance, ask (policy owner) before acting rather than after.

The sentence beginning “Where a return-to-work plan includes monitoring testing” is shown only when the selected testing types include return to work and monitoring.

Supports the (organisation) drug and alcohol policy, version (version), (date). Not legal advice. Prepared with NIVHA Laboratory Services.